



**MEDICAL GROUP
OF KANSAS CITY**

PatientName: Scooby Doo

DOB: 03/06/62

ADVANTRA HMO MEDICARE Payer ID: 25133

Referring: MICHAEL DAHL

Appointment Facility: 212511 MKC THE MEDICAL GRP OF KC

Advance Directive: NAD(03/14/2017)

03/12/2019

PROGRESS NOTE: MICHAEL DAHL MD CHN#: 1124013297

Current Medications

Taking

- Multivitamins as directed Orally once a day
- Aspirin 81 MG Tablet Delayed Release 1 tab(s) Orally daily
- Atorvastatin Calcium 40 Tablet TAKE 1/2 TABLET BY MOUTH ONCE A DAY
- Viagra 100 MG Tablet 1 tablet as needed Orally Once a day
- Tamsulosin HCl 0.4 MG Capsule 1 capsule Orally Once a day
- Losartan Potassium 100 MG Tablet TAKE 1 TABLET BY MOUTH EVERY DAY
- Spironolactone 25 MG Tablet 1 tablet Orally Daily, Notes: Verbal order per provider
- Amlodipine Besylate 10 MG Tablet 1 tablet Orally BID
- Metoprolol Tartrate 50 MG Tablet 1 tab(s) Orally Twice a day
- Klor-Con M20 20 MEQ Tablet Extended Release 1 tablet Orally Twice a day

Discontinued

- Klor-Con M20 20 Unspecified 1 tablet Orally Twice a day
- Medication List reviewed and reconciled with the patient

Past Medical History

- Hypertension.
- BPH.
- Hypokalemia.
- CAD Dr. Deepak.
- Diverticular disease of the colon.
- Lipoma right colon.
- CKD.

Surgical History

Colonoscopy 1997; 1/2009...+ adenomas; 3/2016...+ adenoma; Repeat 3/2019
PTCI/DES in LCX/RCA and LAD 2011
Incarcerated ventral supraumbilical hernia repair. 11/5/18

Reason for Appointment

1. Bp

History of Present Illness

Hypertension:

Here for f/u doing well and without complaints on amlodipine and metoprolol.

Hyperlipidemia:

Here for f/u uncontrolled and thus switched to atorvastatin with last LDL =70 .

CAD/ASCVD:

Here for f/u doing well and without complaints s/p MI and followed by Dr. Deepak and had f/up with next visit in one year.

Erectile dysfunction:

Erectile dysfunction Quality: incomplete erection unable to sustain erection , with Viagra being the only med with slight help.

Renal Insufficiency:

Tests previously showed Cr>2 but last check at 1.67 Reports no problems with UO.

Prostate Cancer:

Prostate Cancer dx'd by Dr. Kuykendal and now some BPH symptoms treated with flomax .

Vital Signs

Ht 69 in, Wt 211 lbs, BMI 31.16, Weight Change 4 lb, BP 132/74, HR 62.

Past Orders

Lab:ICHEM 8+ (RHMC-ISCHEM8) (Order Date - 11/05/2018)

(Collection Date - 11/05/2018)

ISTAT HEMATOCRIT	38.0	34.5-45.0 - % PCV
ISTAT HEMOGLOBIN	12.9	11.5-15.0 - G/DL

Notes: FRANEY,THOMAS A 11/05/2018 03:40:24 PM CST >

Lab:HEMOGLOBIN (RHMC-HGB) (Order Date - 10/03/2018)

(Collection Date - 10/03/2018)

HGB	13.2 L	13.5-16.5 - g/dL
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Notes: FRANEY,THOMAS A 10/13/2018 01:10:54 PM CDT >

Family History

Mother: deceased 49 yrs, accidental death
 Father: deceased 69 yrs
 Brother 1: alive 69 yrs
 Brother 2: deceased 71 yrs, prostate cancer
 Daughter 1: alive 50 yrs
 Son 1: alive 53 yrs

Social History

Alcohol Use Patient does not use alcohol.
 Tobacco Status
 Patient is *a former smoker*
 Exercise: yes.
 Marital Status: Married.
 no Lives with.
 Personal Safety
 Has anyone close to you ever threatened to hurt you? *No*
 Communication Needs
 Do you need glasses or contacts to read brochures? *Yes*
 Do people frequently tell you that you are not hearing and/or understanding them? *No*
 Variations in Communication *none*
 Health Literacy
 Are medication labels often written in a way that is easy to read and understand? *No*
 Do you have problems completing medical forms because of difficulty understanding the instructions? *No*
 High Risk for Sexually Acquired Diseases including HIV
 At Risk: *No*
 Illicit Drug Use
 Patient/Family reports: *No illicit drug use*
 Concerns
 Social Concerns: *No*
 Financial Concerns: *No*
 Living Environment
 Reported as: *House/Condo/Apartment*
 Ambulatory Status
 : *is independent*
 Caffeine: yes, frequency:.
 no Drugs.
 no Diet.
 no Children.
 Occupation/Work: retired.
 no Sexually active.
 Migrated from GE MHVS:
 Former Smoker (1990)
 Alcohol Use - no
 Drug Use - no
 Occupation: Retired
 Regular Exercise - no
 Marital Status-Married
 Children: 2.

Allergies

N.K.D.A.

Hospitalization/Major**Diagnostic Procedure**

see sx above

Review of Systems

Lab: LIPID PANEL WITH REFLEX TO DIRECT LDL (Q-14852)
 (Order Date - 09/17/2018) (Collection Date - 09/17/2018)

Result: In Therapeutic Range
 NON-HDL 91 <130 - mg/dL
 CHOLESTEROL (calc)
 TRIGLYCERIDES 119 <150 - mg/dL
 CHOLESTEROL, TOTAL 128 <200 - mg/dL
 HDL CHOLESTEROL 37 L >40 - mg/dL
 LDL-CHOLESTEROL 70 - mg/dL (calc)
 CHOL/HDL C RATIO 3.5 <5.0 - (calc)

Examination**General Examination:**

Constitutional: alert and oriented, NAD.
 HEENT: Head - NC/AT, PERRLA, clear conjunctiva.
 Neck: no carotid bruit, no JVD.
 Respiratory: clear to auscultation bilaterally, no wheezes non labored.
 Heart: RSR/RRR, normal S1S2.
 Psych: alert, appropriate mood and affect.
 Extremities: trace lower leg edema bilat, pulses 2 plus bilaterally.

Assessments

1. Essential hypertension - I10 (Primary)
2. BPH w/o urinary obs/LUTS - N40.0
3. Other and unspecified hyperlipidemia - E78.5
4. Hypopotassemia - E87.6
5. Atherosclerotic heart disease of native coronary artery without angina pectoris - I25.10
6. CKD (chronic kidney disease) stage 3, GFR 30-59 ml/min - N18.3
7. ED (erectile dysfunction) of organic origin - N52.9
8. Prostate cancer - C61
9. Hyperglycemia - R73.9
10. BMI 31.0-31.9, adult - Z68.31

Treatment**1. Essential hypertension**

Continue Amlodipine Besylate Tablet, 10 MG, 1/2 tab(s), Orally, BID
 Continue Losartan Potassium Tablet, 100 MG, 1 tablet, Orally, Daily

LAB: CREATINE KINASE, TOTAL (CPK) (Q-374)

LAB: HEMOGLOBIN A1c (Q-496)

LAB: LIPID PANEL (Q-7600)

LAB: COMPREHENSIVE METABOLIC PANEL W/EGFR (Q-10231)

Notes: High Blood Pressure: Care Instructions material was published to portal Patient Educated with: Hypertension Facts.pdf (Hypertension Facts.pdf)

Care Plan

Your Treatment and Self-Management Plan discussed today:

General ROS:

Constitutional: Negative for:, fatigue. HEENT: Negative for:, visual changes. Resp/Pulmonology: Negative for:, shortness of air.

Cardiology: Negative for:, chest pain, leg swelling. Genital/Urinary: Positive for:, erectile dysfunction, Negative for:, dysuria.

Your most current Low-density lipoprotein (LDL), or BAD cholesterol is 70 mg/dL; the goal is below 100 mg/dL.

Your current blood pressure is 142/82 mmHg; the goal is below 135/85 mmHg.

Your current weight is 211 lbs; your goal should be _____.

Your exercise goal is 20 minutes of aerobic exercise 3 days a week (moderately brisk walk, bicycle riding, swimming).

To reach your treatment goals, it is important to take all your prescribed medications as instructed. Know the name of your medicine. Check with your provider if you are having trouble with your medicine or decided to stop taking it.

A follow up visit should be scheduled in 6 months.

At your next visit, you can expect:

1. A discussion of your goals and challenges
2. An update on your action plan based on your progress
3. A discussion about whether it is time to visit a dietitian
4. A recheck of your key measures

*******CONFIDENCE SCALE:**

Patient/Care Giver chose 3 on the following 1 - 3 scale.

NOT Confident - SOMEWHAT Confident - VERY Confident -

Additional care management support recommended at this time -No

The above plan was developed collaboratively between the patient and the clinical team member.

Educational Materials and Self-Monitoring Tools provided to the patient.

2. BPH w/o urinary obs/LUTS

Continue Tamsulosin HCl Capsule, 0.4 MG, 1 capsule, Orally, Once a day

3. Other and unspecified hyperlipidemia

Continue Atorvastatin Calcium Tablet, 40 MG, 1/2 tablet, Orally, Once a day

LAB: CREATINE KINASE, TOTAL (CPK) (Q-374)

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4. Hypopotassemia

Continue Klor-Con M20 Tablet Extended Release, 20 MEQ, 1 tablet, Orally, Twice a day

5. Atherosclerotic heart disease of native coronary artery without angina pectoris

Continue Metoprolol Tartrate Tablet, 50 MG, 1 tab(s), Orally, Twice a day

Continue Clopidogrel Bisulfate Tablet, 75, TAKE ONE TABLET BY MOUTH DAILY

Continue Spironolactone Tablet, 25 MG, 1 tablet, Orally, Daily, Notes: Verbal order per provider

6. ED (erectile dysfunction) of organic origin

Refill Viagra Tablet, 100 MG, 1 tablet as needed, Orally, Once a day, 10 day(s), 10 Tablet, Refills 3

7. Prostate cancer

Notes: Per Dr. Kuykendal.

8. Hyperglycemia

LAB: CREATINE KINASE, TOTAL (CPK) (Q-374)

LAB: HEMOGLOBIN A1c (Q-496)

LAB: LIPID PANEL (Q-7600)

LAB: COMPREHENSIVE METABOLIC PANEL W/EGFR (Q-10231)

Preventive Medicine

Quality Measures:

Colorectal Cancer Screening:

Colonoscopy 03/15/2016

Fall Risk Assessment:

Date of Screening Completed: 09/17/2018

Increased Fall Risk Factors: *No fall risk factors*

History Falls in Past Year: *No falls in the past year*

Universal PAF (Patient Assessment Form):

Fall Risk Assessment

Increased Fall Risk factors: *No fall risk factors*

Date Screening Completed 03/12/2018

Male

Prostate Exam: Yes

PSA Level Checked: 09/19/2017

Immunizations

Influenza Vaccine: 10/28/2015

Pneumococcal Vaccine: PCV13

PCV13 Date: 10/01/2017

Universal PAF completed

: Yes

Date 03/14/2017 Faxed: 04/13/2017

PCMH:

Interpreter:

Utilized for visit today: N/A

Utilized for visit today: N/A

Plan and manage care and self-care:

Pre-visit care documented: yes

Pre-visit care documented: yes

Individual care plan and goals reviewed and updated: yes

Medications/education

Barriers to adherence None

Barriers to adherence None

Follow Up

6 Months



Electronically signed by MICHAEL DAHL, MD on
03/13/2019 at 04:15 PM CDT

Sign off status: Completed

313511MKC THE MEDICAL GRP OF KC
6675 HOLMES RD
STE 550
KANSAS CITY, MO 641311167
Tel: 816-363-7710
Fax: 816-363-8414

Patient Name: Scooby Doo DOB: 03/06/62

Note generated by eClinicalWorks EMR/PM Software (www.eClinicalWorks.com)

Apr 26, 2019 13:09 PM

Patient Name: Scooby Doo

DOB: 03/06/62

CUMULATIVE REPORT

CREATINE KINASE, TOTAL (CPK) (Q-374)

COLLECTION DATE	03/12/2019
Order Date	03/12/2019
Result Date	03/13/2019
Ordering Physician	DAHL, MICHAEL
CREATINE KINASE, TOTAL	137 (44-196 U/L)

HEMOGLOBIN A1c (Q-496)

COLLECTION DATE	03/12/2019
Order Date	03/12/2019
Result Date	03/13/2019
Ordering Physician	DAHL, MICHAEL
HEMOGLOBIN A1c	6.0 H ($<5.7\%$ of total Hgb)

LIPID PANEL (Q-7600)

COLLECTION DATE	03/12/2019
Order Date	03/12/2019
Result Date	03/13/2019
Ordering Physician	DAHL, MICHAEL
CHOLESTEROL, TOTAL	118 (<200 mg/dL)
TRIGLYCERIDES	137 (<150 mg/dL)
HDL CHOLESTEROL	34 L (>40 mg/dL)
LDL-CHOLESTEROL	62 (mg/dL (calc))
CHOL/HDL-C RATIO	3.5 (<5.0 (calc))
NON-HDL CHOLESTEROL	84 (<130 mg/dL (calc))

COMPREHENSIVE METABOLIC PANEL W/EGFR (Q-10231)

COLLECTION DATE	03/12/2019
Order Date	03/12/2019
Result Date	03/13/2019
Ordering Physician	DAHL, MICHAEL
GLUCOSE	101 H (65-99 mg/dL)
UREA NITROGEN (BUN)	27 H (7-25 mg/dL)
CREATININE	1.71 H (0.70-1.18 mg/dL)
eGFR NON-AFR. AMERICAN	38 L ($> OR = 60$ mL/min/1.73m ²)
eGFR AFRICAN AMERICAN	44 L ($> OR = 60$ mL/min/1.73m ²)
BUN/CREATININE RATIO	16 (6-22 (calc))
SODIUM	143 (135-146 mmol/L)
POTASSIUM	4.4 (3.5-5.3 mmol/L)

Problem List

Date: 05/01/2019 Age Of Onset: Provider: JP ICD9: 374.87 Dermatochalasis ICD10: H02.831 Dermatochalasis Of Right Upper Eyelid SNOMED CT: 246815009 Excess skin of eyelid (finding) Location: Status: Active Health Status: Comments: Dermatochalasis BUL

Date: 05/01/2019 Age Of Onset: Provider: JP ICD9: 374.87 Dermatochalasis ICD10: H02.834 Dermatochalasis Of Left Upper Eyelid SNOMED CT: 246815009 Excess skin of eyelid (finding) Location: Status: Active Health Status: Comments: Dermatochalasis BUL

Date: 05/01/2019 Age Of Onset: Provider: JP ICD9: 366.12 Incipient Cataract ICD10: H25.093 Other age-related incipient cataract, bilateral SNOMED CT: 445213003 Incipient senile cataract (disorder) Location: Status: Active Health Status: Comments: Incipient Cataract

Chief Complaint

Scooby Doo is a 57 year old male who returns for decreased vision BE distance and near. Pr is prediabetic and on an oral med to help. No discharge, No flashes, No floaters, No Tearing or No pain. PT does have seasonal allergies.

HPI

Reviewed by Dr. Padousis

Location Both Eyes
 Quality Blurred
 Severity Mild
 Duration since last exam
 Timing gradually

Ocular History

Timeline	Type	Location	History	Status	Social
07/05/2011	Family		302891003 -- Macular degeneration (disorder) / Macular Degeneration Grandparent	Active	MH I
07/05/2011	Family		23986001 -- Glaucoma (disorder) / Glaucoma	Active	MH
07/05/2011	Family		54690008 -- Unknown (origin) (qualifier value) / RD	Active	MH

Medical History

Timeline	Type	Location	History	Status	Social
2014	Illness		Hernia	Active	MA
	Other		Have you received a flu vaccine? Yes.	Active	MA

Social History

Timeline	Type	Location	History	Status	Social
	Social		Drinks Social.	Active	MA
	Smoking		Never smoker. / 266919005	Active	MA

Current Ocular Meds

Reviewed JP 05/01/2019

Current Systemic Meds

Reviewed JP 05/01/2019

Allegra 180 mg Tab 180 mg
 Bydureon 2 mg
 Rhinocort Aqua 32 mcg/Actuation Nasal Spray 32 mcg/actuation Add'l Sig Add'l Sig Nasal Add'l Sig
 Crestor 5 mg Tab 5 mg Add'l Sig Add'l Sig Oral Add'l Sig

Allergies

Reviewed by: JP 05/01/2019 NKDA MA 9:10a

Review of Systems

Reviewed by JP

Constitutional No significant
 Eyes See ocular exam.
 ENMT No significant
 Cardiovascular No significant chest pain, palpitations, increased heart rate, or leg edema.
 Respiratory No significant cough, SOB, wheezing, or difficulty breathing.
 Gastrointestinal No significant nausea, vomiting, diarrhea, indigestion, reflux, or constipation.
 Genitourinary No significant painful urination, frequent urination, STD, or kidney ailments.
 Integumentary No significant changes in hair, nails and skin.
 Musculoskeletal No significant joint pain, muscle pain, back pain.
 Neurological No significant change in speech, memory loss, gait disturbances, or loss of coordination.

Hematologic No significant abnormal bleeding, enlarged lymph nodes or swollen glands.
 Immunologic No significant food allergies, seasonal allergies, or immune disorders.
 Endocrine No significant diabetes, hypothyroidism, or hypoglycemia.
 Psychiatric No significant depression, panic disorder, or anxiety problems.

Refractions

DVA sc, OD Rx +1.75 Sph Dva 20/60 +1 Nva J2 17"
 NVA cc OS Rx +1.75 Sph Dva 20/25 +2 Nva J2 22"

Final Rx OD Rx +0.75 +1.50 x 020 Dva 20/20 -1
 OS Rx +0.75 +0.75 x 175 Dva 20/15 -1

Manifest OD Rx +0.75 +1.50 x 020 Add +2.50 Dva 20/20 -1 Nva J1 15"
 OS Rx +0.75 +0.75 x 175 Add +2.50 Dva 20/15 -1 Nva J1 16"

External Exam

Reviewed by JP

	OD	OS
Pupils	Round & Reactive Size 4mm.	Round & Reactive Size 4mm.
Motility	Full	Full
Confrontation Fields	Full	Full
External Lids	dermatochalasis upper lids 1+	dermatochalasis upper lids 1+
Orbit and Adnexa	Normal	Normal
Face	Normal	Normal

IOP

	Target OD	OS	T-Max OD	OS	Pach OD	OS						
Data	Time	Staff	OD	OS	Method	Timing	Adj OD	Adj OS	Gonlo	VF	OCT	Comments
05/01/2019	9:34a	MA	12	11	Goldmann	Examination						
					Applanation							

Drops Administered

Reviewed by JP 05/01/2019

Tropicamide 1% OD Administered at: 9:37a
 OS Administered at: 9:37a
 Neosynephrine 2.5% OD Administered at: 9:39a
 OS Administered at: 9:39a

Slit Lamp Exam

Reviewed by JP 05/01/2019

	OD	OS
Lids and Lashes	Lid Margins Clear, Lashes Without Collarettes or Scurf	Lid Margins Clear, Lashes Without Collarettes or Scurf
Tearfilm	normal tearfilm	normal tearfilm
Conjuncti	Pterygium, nasal.small	Pterygium, nasal.small
	clear	clear
	deep and quiet	deep and quiet
	Normal appearing iris with no signs of neovascularization, atrophy or synechia.	Normal appearing iris with no signs of neovascularization, atrophy or synechia.
	Trace NS.	Trace NS.
	angles appear open	angles appear open

C/D Ratio

Date	05/01/2019	OD Horizontal	OD Vertical	OS Horizontal	OS Vertical	Staff	Comments
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	OD	OS
Dilation	Dilated retinal examination was performed. The patient was warned against driving.	Dilated retinal examination was performed. The patient was warned against driving.
View	Good View	Good View
Optic Discs	Normal appearing optic nerve with healthy pink rim and normal appearing nerve fiber layer.	Normal appearing optic nerve with healthy pink rim and normal appearing nerve fiber layer.
Cup to Discs	Vertical: 0.30.	Vertical: 0.35.
Macula	Normal color and contour for age. No thickening or macular edema.	Normal color and contour for age. No thickening or macular edema.
Vessels	Vessels are of normal caliber without arterio-venous nicking or	Vessels are of normal caliber without arterio-venous nicking

Periphery significant tortuosity
Vitreous Attached 360 Degrees, No Holes or Tears.
Vitreous Normal and Clear for Age, No PVD.

or significant tortuosity
Attached 360 Degrees, No Holes or Tears.
Vitreous Normal and Clear for Age, No PVD.

Diagnosis

Seq	ICD9	Diagnosis	SNOMEDCT	ICD10	Location	Provider	Status	Comments
372.41		Peripheral Pterygium, Stationary	193880000 -- Peripheral pterygium, stationary (disorder)	H11.043 -- Peripheral pterygium, stationary, bilateral		JP	Active	Peripheral Pterygium -- Observe
374.87		Dermatochalasis	246815009 -- Excess skin of eyelid (finding)	H02.831 -- Dermatochalasis Of Right Upper Eyelid		JP	Active	Dermatochalasia BUL
374.87		Dermatochalasis	246815009 -- Excess skin of eyelid (finding)	H02.834 -- Dermatochalasis Of Left Upper Eyelid		JP	Active	Dermatochalasia BUL
366.12		Incipient Cataract	445213003 -- Incipient senile cataract (disorder)	H25.093 -- Other age-related incipient cataract, bilateral		JP	Active	Incipient Cataract
374.87		Dermatochalasis		--		JP	Active	'01/08/15 Dx Last Visit'
372.40		Unspecified Pterygium		--	OU	JP	Active	'01/08/15 Dx Last Visit'

Plan

1. Peripheral Pterygium BE --There is overgrowth of normal elastotic tissue, which extends onto the cornea. small and peripheral with no change
2. Dermatochalasis: Redundant skin of the upper eyelids is present, but not significantly symptomatic. No therapy is needed at present.
3. There are early lens changes in the BE which are not interfering with this patient's visual function. They were not discussed extensively because they are so minimal.
2. Return for an appointment in 2 years for a with Dr. JEFFREY PADOUSIS.

Ocular Medications

No Meds

Reviewed JP 05/01/2019

Patient Education

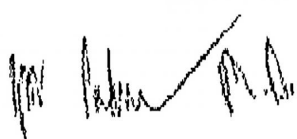
Date	Handout	Staff	Provider	Education	Comments
05/01/2019	Cataract	JP	JP		

Followups

12:00a Followup for a with Dr. JEFFREY PADOUSIS.

Reminders

Setup	Reminder	Type	Order	Interval	Due	Refused	Action
	Return for an appointment in 2 years for a with Dr. JEFFREY PADOUSIS.	Appointments					Scheduled



JEFFREY PADOUSIS,
MD 05/01/2019 10:16
AM

DOCUMENT SIGNED ELECTRONICALLY BY: JEFFREY PADOUSIS, MD 05/01/2019 10:16 AM